

Medanta, Sec-38, Gurgaon

Medanta-The Medicity

REFUND VOUCHER

ORIGINAL

Patient Id	: [REDACTED]	Encounter Id	: 23668618
Patient Name	: [REDACTED]	Refund Date	: 28/12/2025 12:01
Age/Sex	: 27Y 0M 7D / Male	Refund Voucher No.	: GHIPRF/26021976

S.NO	Mode of Payment	Amount
1	Cheque/DD/Pay order (CREDIT CARD-4049, Drawn on: CICI Bank)	-49280.00
Total		-49280.00

INR Forty Nine Thousand Two Hundred Eighty And Paise Zero Only

Refunded to [REDACTED] sum of INR 49280/- towards the bill(s) GHIPBL/26071538.

For Global Health Limited.



Receiver Signature:

Name:

Relationship:

Mobile No:

Checked By

Approved By

Accredited by


JCI certificate
(CN = 36282)

Certificate No
H-2011-0073

For Emergency & Ambulance: Dial @ 1068

Medanta - The Medicity

Gurugram

+ Sector - 38,
28/12/2025 12:03
0124 4141 414

Defence Colony

+ E-18, Defence Colony,
New Delhi
011 4411 4411

Cybercity

+ 10G Floor, Building 10C,
DLF Cybercity, Gurugram, Haryana
0124 414 [REDACTED]

Our NCR Mediclinics

Subhash Chowk

+ Sector - 38, Subhash Chowk
Gurugram, Haryana
0124 4834 547

Golf Course

+ Sector 27, Golf Course
Gurugram, Haryana
0124 6930 099

[REDACTED] Office: Global Health Limited, E-18, Defence Colony, New Delhi, India Tel: 011 4411 4411

info@medanta.org

www.medanta.org

Corporate Identity Number - L85110DL2004PLC128319

Medanta Network: Gurugram | Delhi | Lucknow | Patna | Indore | Ranchi | Noida*

Name: Mr. [REDACTED]
UHID: [REDACTED]
Gender: M
DOB: 21 Dec 1998 (27 years)
Doctor: Dr Deepak Kumar Rathi

Encounter ID: 23668618
Encounter Type: Inpatient
Visit/Admn Date: 23 Dec 2025
Location: 12th Floor A2A3
Speciality: Urology & Andrology

Urology Discharge Summary

Date of Discharge : 28/12/2025
Name of Consultant : Dr Deepak Kumar Rathi
Bed No : 5215
Reason for admissions : Procedure/Surgery
Diagnosis & Co-morbidities :
Diagnosis :

RIGHT RENAL MASS

Allergies :

No known drug allergies

Chief complaints

Procedure or Surgery :

RIGHT ROBOTIC PARTIAL NEPHRECTOMY WAS DONE UNDER GENERAL ANESTHESIA ON 24/12/2025

FINDINGS :

RT MID-POLAR MASS/ENDO-EXOPHYTIC AROUND 4*4 CM
SINGLE RENAL ARTERY AND VEIN
ARTERIAL CLAMP TIME- 19 MIN
CONSOLE TIME- 2" 45 MIN

DESCRIPTION OF PROCEDURE :

LITHOTOMY POSITION
CYSTOSCOPY DONE AND GUIDE WIRE PLACED IN RT PCS
URETERIC CATHETER PLACED UNDER FLUOROSCOPY
PATIENT PLACED IN LEFT LATERAL DECUBITUS POSITION.
PNEUMOPERITONEUM CREATED USING VERESS NEEDLE.
STANDARD RIGHT KIDNEY PORTS MADE, 1 OF 10 MM AND 1 OF 12 MM, 3 OF 5 MM AND 5 MM LIVER RETRACTION PORT.
DA VINCI SI ROBOT DOCKED
RIGHT COLON DEFLECTED ALONG THE WHITE LINE OF TOLDT.
DUODENUM KOCHERIZED AND IVC IDENTIFIED.
HILAR DISSECTION DONE.
SINGLE RENAL ARTERY AND SINGLE RENAL VEIN IDENTIFIED AND DISSECTED.
SUPERIOR POLE OF KIDNEY MOBILIZED AND ADRENAL SPARED.
TUMOR IDENTIFIED ON THE LATERAL SURFACE ON MID POLE OF RIGHT KIDNEY.
RENAL ARTERY CONTROLLED WITH BULLDOG CLAMPS.
MASS EXCISED UPTO NORMAL APPEARING PARENCHYMA.
HEMOSTASIS DONE WITH 3-0 V LOC.
ARTERIAL CLAMP REMOVED.
RENORRAPHY DONE WITH 3-0 PNJ.
HAEMOSTASIS SECURED.
JACKSON PRATT DRAIN PLACED.
ABDOMEN DESUFFLATED.
SPECIMEN RETRIEVED FROM EXTENDED 12 MM PORT.

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PORTS REMOVED UNDER VISION.
PORT SITES CLOSED.

Medical History & Presenting Complaints

Medical History & Presenting Complaints :

Mr. [REDACTED] 27 years old gentlemen has come with right renal mass.
Admitted for Right Robotic Partial Nephrectomy.

Physical & Systemic Examination

Physical & Systemic Examination :

Conscious, oriented
No pallor / cyanosis / clubbing / icterus / lymphadenopathy / raised Jugulo venous pressure
Afebrile

Pulse- 79/min; Respiratory rate- 20/min; Blood pressure- 140/90 mmHg
Cardiovascular system- S1 & S2 normal, no added sounds
Chest- Bilateral clear

Per abdomen- Soft, BS present, no organomegaly, no tenderness
Central nervous system Within normal limits

Course in Hospital

Course in Hospital :

Patient was admitted under Dr Deepak Kumar Rathi. Patient was thoroughly investigated on OPD basis. Clearance was obtained from Cardiology/Pulmonology.
Patient and his relatives were given a comprehensive overview about surgery, the possible benefits & complications; about the process of admission, duration of stay, the procedure itself, likely outcome and possible complications (include, but are not limited to: infection/ bleeding/ pain/ rupture or slipping way of clamps on vessels during clamping of vessels/ chances injury to intra-abdominal or adjacent organs- bowel, nerves, muscles, liver, pancreas and gall bladder/lung cavity puncture, pneumothorax or air embolus/incisional hernia/urinary leaks/hematoma formation/deep vein thrombosis or thromboembolism/ conversion to open surgery, etc. The consent taken includes partial/ complete removal of kidney in case of bleeding or any other complications that would jeopardise the functioning of the kidney or endanger life itself. In such case of profuse bleeding and threat to life, procedure may be converted to open or a radical removal of the whole kidney. Right Robotic Partial Nephrectomy was done under General Anesthesia on 24/12/2025. The patient tolerated the procedure well. The post- operative period was uneventful. The patient's relatives were informed post surgery, meeting the patients in the post operative room (recovery) and further line of management explained in detail. Oral liquids supplemented with intravenous nutrition; Patient controlled analgesia (PCA) pumps gives them an upper hand, which is continued upto the next post operative day. Patients are encouraged to mobilise their limbs and reassured about change in position on the same operative day. Foley's catheter was removed on 26/12/2025 and the patient is voiding well with good flow and control. The patient is ambulatory (cautioned and demonstrated how to help themselves while getting out of bed- to prevent falls/injuries) commencing from the first post operative day, active, improving pain control (analgesics evenly distributed round the clock) with improving bowel movement; active physiotherapy & incentive spirometer with gradually tolerating small meals (improving appetite). Drain was removed on 27/12/2025. Dressing was done; wound status was good. They were explained about wound care/dressing, daily bath, other precautionary measures at home and subsequent follow up. The patient is now being discharged in stable condition with the following advice.

Significant Medication Given :

Injection Magnex Forte

Condition at Discharge

Condition at Discharge :

Stable

Investigations

Advise on Discharge

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UHID: [REDACTED]

Gender: M

DOB: 21 Dec 1998 (27 years)

Doctor: Dr Deepak Kumar Rath

Encounter ID: 23668618

Encounter Type: Inpatient

Visit/Admn Date: 23 Dec 2025

Location: 12th Floor A2A3

Speciality: Urology & Andrology

Discharge Medication

Discharge Medication				
Drug	Dose	Drug Class	Frequency	Remark
Tablet CEFTUM	500mg		Twice Daily	for 5 days
Tablet PANTOCID	40mg		Empty stomach	for 5 days
Tablet DOLO	650mg		Twice Daily	for 2 days then as and when required for pain and fever
Syrup LOOZ	30ml		At Bedtime	for constipation

Diet

As advised

Discharge Instructions

To continue all medications for other comorbidities as earlier

Steam inhalation thrice daily for 1 week

Incentive Spirometry as advised for 2 weeks

Avoid constipation

Avoid heavy meals; Prefer small frequent meals as tolerated for the first week

Good bed rest for about 7-10 days (can perform daily normal activities)

Safe to return to basic everyday activities in 2 weeks

Avoid activities that put strain on the groin or lower abdominal area for 4 weeks

May return to strenuous activities/ lifting heavy objects/ vigorous activity for another 1-2 months

Care of the surgical sites (keep open and dry)

Can take showers

Chances of discharge from surgical site (likely serous, may use pad for absorption)

Pain may be bothersome upto 2 weeks (use analgesics)

Following conditions are not an emergency or be alarmed

-Chances of bleeding/discharge /collection from the site of surgery

-Chances of intermittent hematuria

-Chances of low grade fever (T < 100°F) for 1-2 weeks (report if > 101°F)

-Chances of intermittent pain at the flank/incision site (advised to continue analgesics as long as required)

When is it an Emergency?

-Report if fever is > 101°F (with chills)

-reduce urine output

-blood in urine- gross with clots

-dizziness, shortness of breath or confusion

-severe or intensifying abdominal pain or discomfort

-abdominal distension

Plan:

-Review in Urology Outpatient Department, 12th Floor, Dr. Deepak Kumar Rath with prior appointment on Friday(2/1/2026) with HPE reports

WHEN TO OBTAIN URGENT CARE

In case of any problem like:

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- FEVER (T>100 degrees) with chills
- Severe Pain not controlled by oral pain killers
- Blood Clots/Dark Blood tinged Urine.
- Reduced Urine output.
- Rash over skin , generalized body swelling.
- Chest Pain , breathing difficulty, profuse sweating, giddiness, pain in abdomen

Contact Urology Duty mobile number (0124-4834512 / +91 9560398928) / MEDANTA- THE MEDICITY : 0124-44114411
Or any other medical problem for which you think urgent attention is required report to emergency at Medanta-The Medicity at the earliest possible. (0124-4141414, Ext.No. 2404 & 2406).

Follow up :

To follow-up in Urology Outpatient Department, 12th Floor, Dr. Deepak Kumar Rathi with prior appointment (Contact Mr. Mohit Uniyal +9560398966, 0124-4141414).

Access your medical reports and follow up with Doctors through video conferencing by downloading Medanta eCLINIC App or by visiting www.medanta.eclinic.org

Activate your eCLINIC account using Medanta Patient UHID [REDACTED]

For any assistance or query call +124-4855017 or write to eclinic@medanta.org

FOR EMERGENCY AND AMBULANCE PLEASE CONTACT: 1068.

Medanta Care at Home provides services in the comfort of your home.

- Sample Collection
- Nursing Visits
- Medicine Delivery
- Child and Adult Vaccination

To book, Call +91 124 4834565 or Whatsapp 8130771414.

Authorized by: Dr Aayush Jain

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